

CONSULTATION NOTE — PULMONOLOGY & ALLERGY OPD

Patient	Ananya Krishnan	Date	05-Mar-2025
Age/Sex	34 Years / Female	Visit	Follow-up (6-month post-discharge)
Consultant	Dr. Suresh Pillai, MD Pulmonology	Peak Flow	410 L/min
Weight	61 kg	SpO2	99% room air

SUBJECTIVE

Patient reports significant improvement in asthma control since starting Budesonide/Formoterol combination inhaler. Last acute exacerbation was the hospital admission in August 2024 (7 months ago). No ER visits since discharge. Rescue inhaler use: 1-2 puffs per week (was 6-8 puffs/day at admission). Mild rhinitis symptoms persist in mornings. Anxiety well-controlled on Escitalopram — reports better sleep and mood. No new drug reactions or side effects reported.

CURRENT MEDICATIONS — VERIFIED

Medication	Dose	Compliance	Notes
Budesonide/Formoterol inhaler	200/6mcg 2 puffs BD	Good	Controller — continue
Salbutamol inhaler	100mcg SOS	Good	Rescue — using 1-2x/week only
Montelukast	10mg ON	Good	Leukotriene antagonist
Escitalopram	10mg OD	Good	Anxiety — psychiatry managed
Loratadine	10mg OD	Intermittent	For rhinitis — takes as needed

OBJECTIVE

- Peak Flow Rate: 410 L/min (82% predicted, was 52% at admission) — significant improvement
- SpO2: 99% room air
- Chest auscultation: Clear, no wheeze, good air entry bilaterally
- Nasal examination: Mild pale boggy turbinates — allergic rhinitis
- No skin findings. No angioedema.

SPIROMETRY TODAY

Parameter	Aug 2024 (Acute)	Jan 2025	Mar 2025	Status
FEV1	—	2.18 L (76%)	2.42 L (85%)	IMPROVING
FVC	—	2.95 L (86%)	3.18 L (93%)	IMPROVING
FEV1/FVC	—	0.74	0.76	BORDERLINE — improving
Peak Flow	210 L/min (52%)	—	410 L/min (82%)	GOOD RESPONSE

Spirometry showing consistent improvement. Mild-moderate obstructive pattern remains — expected in persistent asthma. Well-controlled on current regimen.

ALLERGY REVIEW

CONFIRMED ALLERGIES — must be on all prescriptions:

1. Aspirin / NSAIDs — triggers severe bronchospasm (Samter's triad). ABSOLUTE CONTRAINDICATION.

2. House Dust Mite, Cat Dander, Cockroach — environmental triggers documented on IgE panel.

Avoidance measures in place: HEPA filter at home, dust mite covers on bedding, no pets.

ASSESSMENT

1. Bronchial Asthma — well-controlled (GINA Step 3). Excellent response to ICS/LABA.
2. Allergic Rhinitis — mild persistent. Undertreated — need to optimise.
3. Anxiety Disorder — well-controlled on Escitalopram. Psychiatry to continue.
4. Aspirin/NSAID sensitivity — confirmed. All treating doctors must be aware.

PLAN

- Continue Budesonide/Formoterol 200/6mcg — do not step down yet
- Add Mometasone nasal spray for allergic rhinitis — once daily
- Consider allergen immunotherapy (sublingual SLIT for dust mite) in 3 months if rhinitis persists
- Annual influenza vaccine — recommended for asthmatic patients
- **Medical alert card issued: Aspirin/NSAID anaphylaxis risk. Use Paracetamol for all pain/fever.**
- Next pulmonology review in 6 months or sooner if exacerbation occurs
- Asthma action plan updated and given to patient